



PhilHealth
Your Partner in Health



**DEPARTMENT OF HEALTH
PHILIPPINE HEALTH INSURANCE CORPORATION
DEPARTMENT OF INFORMATION AND COMMUNICATIONS TECHNOLOGY**

AUG 11 2021

JOINT ADMINISTRATIVE ORDER

No. 2021- 0001

SUBJECT: Guidelines on the Implementation of the Standards Conformance and Interoperability Validation

I. RATIONALE

Pursuant to Section 36.1 of the Implementing Rules and Regulations (IRR) of the Universal Health Care (UHC) Act, one critical strategy identified to improve the country's health information system is through interoperability of health services and information flow across geographical and health sector boundaries. This can be achieved through adoption and mandatory use of common health data standards. The Department of Health (DOH) and the Philippine Health Insurance Corporation (PhilHealth) have defined the mandatory national health data standards for interoperability, and compliance with these standards is now needed.

This Joint Administrative Order sets the Standards Conformance and Interoperability Validation to enable different Health Information Systems (HIS) and other eHealth solutions of health care providers, insurers, and health-related entities to seamlessly communicate, share, exchange, process and submit health and health-related data and reports at the local (i.e. within and across province-/city-wide health system [P/CWHS]) and national levels (i.e. to PhilHealth and/or DOH).

II. OBJECTIVES

This Order institutionalizes the implementation of Standards Conformance and Interoperability Validation (SCIV) as a compliance monitoring mechanism to achieve an integrated and interoperable health and health-related data and report processing and submission at all levels of healthcare utilization.

Specific objectives of this issuance are as follows:

- A. Describe the methods and processes involved in conducting SCIV that will ensure conformance with national health data standards for interoperability, and compliance with rules on processing and submission of health and health-related data and reports through the issuance of the SCIV Toolkit; and
- B. Define the implementation governance that will direct, coordinate, and guide various stakeholders involved in SCIV.

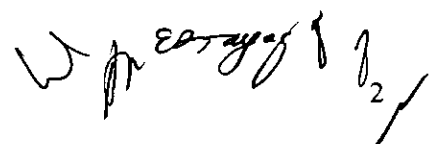
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III.SCOPE OF APPLICATION

This Order shall apply to all public and private, national and local health care providers and insurers which shall be required to process and submit health and health-related data for local and national health data reporting to DOH and/or PhilHealth using their integrated HIS and/or any other eHealth solution; other health-related entities; all national, regional, local and branch offices under the DOH, PhilHealth, and Department of Information and Communications Technology (DICT); Ministry of Health – Bangsamoro Autonomous Region for Muslim Mindanao (MOH-BARMM); other national government agencies (NGAs) involved in the processing and submission/sharing of health and health-related data and reports to DOH and/or PhilHealth; local government units (LGUs); and all others concerned.

IV.DEFINITION OF TERMS

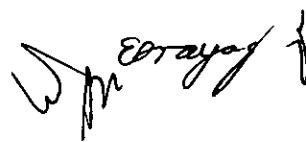
- A. **Applicant** refers to any individual or institutional health care provider, insurer, or any health-related entity that seeks SCIV of their application software for compliance with national health data standards for interoperability, and rules on processing and submission of health and health-related data and reports.
- B. **Application Software** refers to a type of computer program that performs a specific function.
- C. **Centralized Online Standards Conformance and Interoperability Validation System** refers to the national online management information system for registration, verification, and conduct of the SCIV testing.
- D. **eHealth solutions** refer to digital health services, products, systems and applications, networks and technologies that are being used by health care providers, insurers or health-related entities for local and national health and health-related data reporting to DOH and PhilHealth, and linking members of the primary care provider network (PCPN) to secondary and tertiary care providers within and across P/CWHS. This includes HIS, mHealth and telehealth applications, etc.
- E. **Health and Health-Related Data** collectively refer to a set of specific variables or parameters that relates to individual and population health and well-being, including, but not limited to, administrative and investment planning in health, public health, medical, pharmaceutical, and health financing data.
- F. **Health-Related Entities** refer to municipal/city, regional and provincial health offices, academic and research institutions, civil society organizations, medical societies, health professional associations, non-government organizations, donor or funding agencies, development partners, and local and international ICT service providers which shall either develop, implement, sell, or distribute HIS and/or other eHealth solutions, or provide technical assistance or support to implementation and maintenance of integrated HIS, and all other stakeholders involved in the collection, processing and submission of health and health-related data and reports, and/or providing health services in the Philippines, and those identified by the DOH and PhilHealth.
- G. **Insurers** refer to local health insurance offices of PhilHealth, health maintenance organizations and private health insurance companies issued certificates of authority by the Insurance Commission, and those identified by DOH and PhilHealth.



- H. **Integrated Health Information System** refers to an integrated automated system for communication and processing of health and health-related data and reports that are needed for operations and delivery of individual- and population-based health services, response to health emergencies and events of public health concern, health policy development, decision-making, and program planning and implementation at all levels of healthcare utilization. This includes electronic health/medical records, laboratory and diagnostics, electronic prescription and dispensing, telemedicine, human resource information, clinical decision support, referral system management, electronic claims processing and provider payment, enterprise resource planning, supply chain management, including electronic health commodities logistics, financial and capital asset management, quality management systems, and other related services and systems as required by DOH and PhilHealth.
- I. **Interoperable/Interoperability** refers to the ability of different systems to process, share, and exchange data cohesively.
- J. **National Health Data Category Standard** refers to a specific health business process or service. It can also be referred to as use case. Examples of national health data category standard are disease surveillance and response, primary care benefit package, and specific public health programs, etc.
- K. **National Health Data Standards** refers to a set of standardized health or health-related terminology, definition and structure for interoperability.
- L. **SCIV Toolkit** refers to the document, which can be downloaded through the Centralized Online SCIV System that shall be established by the DOH, PhilHealth, and DICT, that provides for the process and method guide, requirements checklist, technical specifications/scripts, and standards references for the conduct of SCIV.
- M. **SCIV Testing** refers to the process of ensuring that an HIS and/or any eHealth solution conforms with national health data standards for interoperability, and complies with rules on processing and submission of health and health-related data and reports both at the local and national levels.

V. GENERAL GUIDELINES

- A. All health information systems and other eHealth solutions that are being implemented or will be implemented by health care providers, insurers, and health-related entities shall be required to undergo and pass SCIV to verify conformance with national health data standards for interoperability, and compliance with rules on processing and submission of health and health-related data and reports at the local (i.e. within and across province-/city-wide health system [P/CWHS]) and national levels (i.e. to PhilHealth and/or DOH).
- B. The SCIV shall be institutionalized as a fair and transparent compliance-monitoring mechanism for integrated and interoperable health and health-related data and report processing and submission for all health information systems and other eHealth solutions, regardless of type of system development (i.e. in-house developed vs. outsourced), nature of ownership (i.e. government-owned vs. private-owned), and stage of system implementation (i.e. for deployment vs. ongoing implementation). It shall also serve as a requirement for the


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establishment of integrated health information systems at all levels of health care, public and private.

- C. The SCIV Toolkit shall provide mandatory guidance to all health care providers, insurers, and other health-related entities on the methods, processes, and standard requirements involved in the conduct of SCIV.
- D. The SCIV implementation shall be an interagency partnership among the DOH, PhilHealth, and DICT, in consultation and coordination with health care providers, insurers, health-related entities, and other relevant stakeholders.
- E. Declaration and management of conflict of interest (COI) shall be mandatory for all individuals and teams involved in the SCIV implementation. All stakeholders involved shall comply with existing government guidelines on the management of COI.

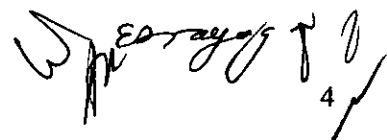
To uphold fairness and transparency in the conduct of SCIV and foster good governance, applicants are prohibited from actively engaging with the Regional SCIV Administrators unless their inputs are sought.

- F. In case of an event of public health concern such as public health emergency, an expedited SCIV process may be implemented upon the receipt of a directive or written letter from the Secretary of Health (SOH).
- G. The implementation of SCIV shall be reviewed, monitored and evaluated periodically by DOH, PhilHealth, and DICT, in consultation and coordination with health care providers, insurers, health-related entities, and other relevant stakeholders, to ensure its alignment with UHC Act and other health- and ICT-related laws, new policies and plans relating to local and national health and health-related data reporting from the source to the PhilHealth and/or DOH, emerging directives for local and national health systems integration, and other emerging international standard approaches and developments relating to software validation.

VI. SPECIFIC GUIDELINES

A. Implementation Governance

- 1. The Secretary of Health (SOH), President and Chief Executive Officer (PCEO) of PhilHealth, and Secretary of Information and Communications Technology (SICT) shall provide overall policy directions and oversight relating to the SCIV implementation.
- 2. The DOH, PhilHealth, and DICT shall collaborate with health care providers, insurers, health-related entities and other relevant stakeholders in the planning and adoption of ICT best practices, standards, and approaches to software validation.
 - a. The DOH, PhilHealth, and DICT shall act as the lead administrators of the SCIV.
 - b. The DICT shall also act as the provider of technical expertise on ICT best practices and standards for software validation.
 - c. The health care providers, insurers, and health-related entities shall act as the core implementers of the SCIV.
- 3. The Health Facilities and Infrastructure Development Team (HFIDT) through the Knowledge Management and Information Technology Service (KMITS) of the DOH, the Information Management Sector (IMS) of PhilHealth, and the Management Information



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Systems Service (MISS) of the DICT shall constitute the SCIV Technical Working Group (TWG) that shall define the implementation strategies and standards, and oversee and monitor the SCIV implementation through a bi-annual assessment in accordance with the policy directions of the SOH, PCEO, and SICT.

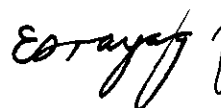
4. The DOH, PhilHealth, and DICT shall designate Regional SCIV Administrators who shall implement the SCIV process, and provide administrative, coordination, and technical assistance or support to all applicants. Appropriate personnel orders shall be issued by the DOH Centers for Health Development (CHD), PhilHealth Regional Office (PROs), and DICT Regional Office (ROs) to this effect.
5. To foster transparency and objectivity, the Regional SCIV Administrators shall be required to invite at least two (2) observers to attend and witness the conduct of the actual SCIV testing. The observers shall come from any national government agency, non-government organization, academic or research institution, or other private entity, provided that these individuals hold no COI.
 - a. All observers shall be subject to the same COI disclosure and COI management requirements similar to all individuals and teams involved in the SCIV implementation.
 - b. The non-attendance of these observers does not invalidate the actual testing, provided that they have been formally invited at least five (5) working days prior to the scheduled testing.

B. SCIV Operations

1. **SCIV Framework.** All health care providers, insurers, and health-related entities that implement or will implement an integrated HIS and/or other eHealth solution, regardless of the type of system development, nature of ownership, and stage of system implementation, shall be required to subject their application software to undergo and pass SCIV for:
 - a. Conformance with national health data standards for interoperability; and
 - b. Compliance with rules on processing and submission of health and health-related data and reports at the local (i.e. within and across P/CWHS) and national levels (i.e. to PhilHealth and/or DOH).
2. **SCIV Testing Methodology.** An SCIV testing methodology shall be developed, implemented and maintained by the DOH, PhilHealth, and DICT following ICT best practices and standards on software validation, and shall be reflected in the SCIV Toolkit.
3. **How to Initiate the SCIV Process.** Application can be done in two (2) ways – i.e. through (a) the Centralized Online Standards Conformance and Interoperability Validation System, or (b) submission of an accomplished SCIV Application Form, together with other minimum documentary requirements, through their respective Regional SCIV Administrators starting 1 October 2021.

No cost shall be charged for SCIV applicants. Should there be a need to charge application cost in the future for operational maintenance, the cost shall be subject for review and approval by the DOH, PhilHealth, and DICT.

4. **Suspension or Pre-Termination of the SCIV Application.** Failure on the part of the applicant to comply or submit any of the SCIV requirements within five (5) working days upon notice shall constitute sufficient grounds to forthwith suspend, or terminate its SCIV



application. Issuance of such notices shall be based on the non-compliance and corrective action procedure that shall be released by DOH and PhilHealth.

5. **Applicable Presumptions.** The Regional SCIV Administrators shall presume that the applicant acted in good faith and with due diligence in following the SCIV process relative to their submissions, representations, and other compliance requirements for SCIV.
6. **Representation and Warranty.** By applying through the Centralized Online SCIV System, or filing and submitting the documents to their respective Regional SCIV Administrators pursuant to this Order, the applicant duly warrants and represents that the documents so filed and registered are genuine, complete, unaltered, and exact copies of the original, and that the contents thereof, and all other information so communicated in relation to their SCIV application, are true and correct.

Any violation of this provision shall constitute sufficient grounds for the Regional SCIV Administrators to suspend or terminate its application, nullify the SCIV results, without prejudice to the possible filing of appropriate administrative, civil, or criminal cases, or a combination thereof, as may be warranted under applicable laws, rules, and regulations.

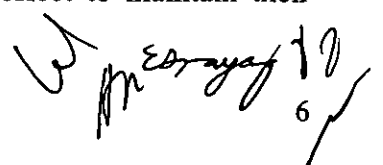
7. **Conduct of SCIV Testing.** The DOH, PhilHealth, and DICT, through the Regional SCIV Administrators, shall:
 - a. Open and maintain a test site/environment where applicants can test their application software, free of charge, prior to undergoing the actual SCIV testing; and
 - b. Implement the SCIV testing methodology, including checking and verification of test results with observers, if available.
8. **SCIV Results and Recommendations.** Upon completion of the SCIV testing, the Regional SCIV Administrators shall notify the applicant of the SCIV results with recommendations within three (3) working days.

A certificate shall also be issued to applicants whose application software has passed the SCIV testing as proof that verification has been completed, and the minimum compliance requirements for health and health-related data and report processing and submission have been met. The certificate shall remain valid unless revoked, suspended, or new amendments or updates on the SCIV framework, and/or internal changes in their software specifications have been instituted.

9. **Appeal.** If the applicant does not agree with the SCIV results, he/she can appeal the decision of the Regional SCIV Administrators either through (a) the Centralized Online SCIV System, or (b) submission of an accomplished SCIV Appeal Form, together with the SCIV Test Report, through their respective Regional SCIV Administrators.

Upon receipt, the appeal shall be acted upon by the SCIV TWG or its authorized adjudicator(s) with the appeal decision issued to the appellant within five (5) working days.

10. **Amendments or Updates.** Amendments or updates to the SCIV framework, particularly on the requirements checklist, technical specifications/scripts, standards references, and testing methodology shall require re-subjection of application software to SCIV by concerned health care providers, insurers, and health-related entities with issued certificate within three (3) months from the date such changes take into effect to maintain their

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compliance status. They shall likewise be required to re-submit their application software to SCIV within the same period if there are significant internal changes in their software specifications that have been implemented.

11. **Revocation of SCIV Certificate.** The Regional SCIV Administrators shall suspend or revoke the certificate for those that have committed violations that threaten or undermine the integrity of the SCIV process, or failure to process and submit health and health-related data and reports at the local and national levels, or any form of misrepresentation of the DOH, PhilHealth, and/or DICT.

C. Monitoring

1. Monthly monitoring shall be conducted by Regional SCIV Administrators using method, tools, and performance indicators as set by the SCIV TWG.
2. Monitoring results shall serve as basis for annual advisory updates that shall be issued by the SCIV TWG every January of the succeeding year.

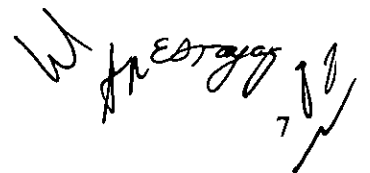
D. Evaluation

1. Evaluation shall be performed every two (2) years by the SCIV TWG based on the policy directions of the SOH, PCEO, and SICT on the scope and method of execution.
2. Evaluation results shall be used to determine if the program is effective in attaining its goals and objectives in relation to UHC Act, and consequently, serve as basis or reference for improvements in policy and program implementation. Accordingly, new policy and programmatic updates shall be issued every January of the following year.

VII. ROLES AND RESPONSIBILITIES

A. Department of Health, Philippine Health Insurance Corporation, and Department of Information and Communications Technology

1. The HFIDT, through the KMITS, PhilHealth IMS, and DICT MISS shall:
 - a. Oversee the SCIV implementation, including setting of implementation strategies and standards, assessment and monitoring of progress status of implementation, and policy and program evaluation, in accordance with the policy directions from the SOH, PCEO, and SICT;
 - b. Establish coordination and networking mechanism with health care providers, insurers, health-related entities, and other relevant stakeholders in planning, monitoring and evaluation of the SCIV implementation;
 - c. Develop, implement, and maintain the Centralized Online SCIV System;
 - d. Define, review and update the content of the SCIV Toolkit as necessary;
 - e. Distribute, disseminate and/or publish the approved SCIV Toolkit through the DOH, PhilHealth, and DICT websites, DOH CHDs, PROs, and DICT ROs, or other means as shall be agreed upon by DOH, PhilHealth, and DICT; and
 - f. Conduct researches to constantly improve the SCIV implementation, and facilitate capacity-building and policy-development initiatives.
2. The CHDs, PROs, and DICT ROs, through the Regional SCIV Administrators, shall:
 - a. Implement the SCIV process as stipulated in this Order and any subsequent updates or changes hereto;
 - b. Administer and manage the SCIV testing, subject to existing administrative, privacy, security and other technical requirements;

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- c. Submit monthly SCIV implementation report within their respective regions to the SCIV TWG;
- d. Provide technical assistance or support to all applicants within their respective regions; and
- e. Manage issues, conflicts, and risks arising from the SCIV implementation.

B. Health Care Providers, Insurers and Health-Related Entities shall:

- 1. Comply with the guidelines as stipulated in this Order and any subsequent updates or changes hereto;
- 2. Subject the application software of their HIS and/or other eHealth solution for local and national health and health-related data and report processing and submission to SCIV;
- 3. Implement only HIS and/or other eHealth solution that are SCIV-compliant.
- 4. Ensure availability of needed investments and support mechanisms to comply with the SCIV results and recommendations, if applicable;
- 5. Ensure the integrity, security, and confidentiality of health and health-related data and reports being processed, maintained, transmitted, or exchanged through their HIS and/or other eHealth solution, uphold the rights of the data subjects, adhere to general data privacy principles, and observe the requirements for lawful processing, pursuant to RA 10173 (Data Privacy Act of 2012) and other applicable Philippine laws and regulations;
- 6. Report issues, concerns, and/or problems that may arise in the subsection of their application software to SCIV to the Regional SCIV Administrators for appropriate action; and
- 7. Provide relevant inputs and feedback to further improve the SCIV implementation.

VIII. PENALTY CLAUSE

Penalties and sanctions for non-compliance with the provisions of this Order and its subsequent guidelines such as non-renewal of license to operate, revocation of service rights and privileges, and/or non-processing and release of Special Health Fund, or PhilHealth electronic claims and/or other related resources despite issued notices shall be imposed, in accordance with the issuance on non-compliance and corrective action procedure that shall be released by DOH and PhilHealth.

IX. BUDGET REQUIREMENTS

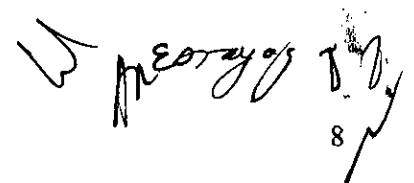
The DOH, PhilHealth, and DICT shall separately allocate funds and provide counterpart resources necessary and appropriate to the overall and regularly funded functions of each agency for the proper implementation of this Order. Each agency shall secure the Commission on Audit's Post Audit review over any and all transactions related hereto.

X. TRANSITORY PROVISION

Within six (6) months from the effectivity of this policy, all responsible DOH, PhilHealth and DICT offices shall release new, updated, or supplemental issuances, and operationalize the provisions of this issuance.

XI. REPEALING CLAUSE

All previous issuances whose provisions are inconsistent with or contrary to any of the provisions of this Joint Administrative Order are hereby rescinded and modified accordingly.

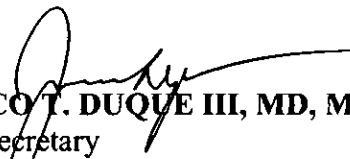


XII. SEPARABILITY CLAUSE

In the event that any provision or part of this Joint Administrative Order is declared unauthorized or rendered invalid by any Court of Law or competent authority, those provisions not affected by such declaration shall remain valid and in force.

XIII. EFFECTIVITY

This Joint Administrative Order shall take effect after fifteen (15) days following its complete publication in a newspaper of general circulation, and upon filing three (3) certified copies to the University of the Philippines Law Center.


FRANCISCO T. DUQUE III, MD, MSc
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